



OFFICIAL

Standard and Transmission Based Precautions Procedure

1. Applicability

Site	Operational Area	Applicable to
Armadale Kalamunda Group	All areas	All workforce groups

2. Purpose

This document aims to define standard precautions and transmission-based precautions and provide details on the application of these precautions for the prevention and management of infections to patients and healthcare workers.

3. Glossary

The following definitions are relevant to this policy.

Term	Definition
Aerosol-generating procedure (AGP)	Procedure that promotes the generation of fine airborne particles (aerosols) that may result in the risk of airborne transmission of disease.
Fit checking	A fit check must be performed each time a PFR is used to ensure it is properly applied, that a good seal is achieved over the bridge of the nose and mouth, and there are no gaps between the face and respirator.
Fit testing	The use of a qualitative or quantitative method of ensuring the correct PFR is matched to the individual's face enabling a good seal to be achieved.
Negative pressure isolation room (NPIR)	A room in which the air pressure differential between the room and the adjacent indoor airspace directs the air flowing into the room i.e. room air is prevented from leaking out of the room and into adjacent areas such as the corridor.
Particulate Filter Respirator (PFR)	Respirators that filter at least 94 percent of 0.3-micron particles from the air. PFRs are used when implementing airborne precautions. Both P2 and N95 respirators are appropriate for use with airborne precautions.
Personal protective equipment (PPE)	A variety of barriers used alone or in combination to protect mucous membranes, skin, and clothing from contact with infectious agents. PPE may include gloves, masks, respirators, protective eyewear, and face shields and gowns/aprons.

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Aerosol-generating procedure (AGP)	Procedure that promotes the generation of fine airborne particles (aerosols) that may result in the risk of airborne transmission of disease.
Fit checking	A fit check must be performed each time a PFR is used to ensure it is properly applied, that a good seal is achieved over the bridge of the nose and mouth, and there are no gaps between the face and respirator.
Fit testing	The use of a qualitative or quantitative method of ensuring the correct PFR is matched to the individual's face enabling a good seal to be achieved.
Standard precautions	Work practices that constitute the first-line approach to infection prevention and control in the healthcare environment. These are recommended for the treatment and care of all patients.
Transmission-based precautions	Measures used in addition to standard precautions when the suspected or confirmed presence of infectious agents presents an increased risk of transmission.

4. Standard Precautions

Standard precautions are the minimum set of practices used in healthcare settings to prevent the transmission of infection from one person to another by providing protection from exposure to blood and body fluids. Standard precautions apply to everyone, regardless of their known or presumed infectious disease status.

Standard precautions consist of the following:

4.1 Hand hygiene

Hand hygiene is the single most important strategy in preventing healthcare associated infections (HAIs)

All staff must perform hand hygiene in accordance with the 5 Moments for Hand Hygiene. Refer to [AKG Hand Hygiene Policy](#)

4.2 Safe use and disposal of sharps

The use of sharp devices exposes HCWs to the risk of injury and potential exposure to blood borne infectious agents. Sharp injuries can occur in any healthcare setting, including non-hospital settings such as Home Hospital. All HCWs must take precautions to prevent injuries caused by needles, scalpels and other sharp instruments or devices.

Refer to [AKG Occupational Exposure to Blood and Body Fluids Procedure](#)

4.3 Routine cleaning of environment

Transmission of infectious agents from the environment to patients may occur through direct contact with contaminated equipment, or indirectly, via hands that are in contact with contaminated equipment or the environment and then touch a patient.

For further information on cleaning procedures, refer to [AKG Cleaners Manual](#).

Blood and body fluid spills must be promptly cleaned. Refer to [AKG Blood and Body Fluid Spills Management Procedure](#).

4.4 Reprocessing of reusable medical equipment and instruments

Transmission of infectious agents from the environment to patients may occur through direct contact with contaminated equipment, or indirectly, via HCW contaminated hands.

Shared clinical equipment must be decontaminated between each patient use using the hospital approved disinfectant wipe.

All staff have a responsibility to clean patient care equipment.

Refer to [AKG Cleaning and Disinfection of Reusable Clinical Non-Invasive Equipment Procedure](#)

4.5 Aseptic technique

Aseptic technique (AT) is a set of practices during clinical procedures aimed at minimising contamination and protecting patients from infection.

Refer to [AKG Aseptic Technique Procedure](#)

4.6 Waste management

Waste should be contained in the appropriate receptacle and disposed of according to the facility waste management plan, refer to [EMHS Waste Management Policy](#).

For cytotoxic waste management, refer to [AKG Safe Handling and Waste Management of Hazardous Medications Guideline](#).

4.7 Appropriate handling of linen

Linen should be handled in such a way to prevent contamination of HCWs clothing and transfer of microorganisms to other patients and the environment.

Refer to [AKG Linen Management Policy](#)

4.8 Personal Protective Equipment (PPE)

PPE refers to a variety of barriers, used alone or in combination, to protect mucous membranes, airways, skin and clothing from contact with infectious agents. PPE may include aprons, gowns, gloves, surgical masks, protective eyewear and face shields. The effectiveness of PPE in preventing exposure to the infection hazard is dependent on access to and correct use of PPE appropriate to the risk of transmission of infectious agents.

4.8.1 Glove use

Gloves are used to prevent contamination of healthcare workers' hands and are indicated for use when there is potential for exposure to blood or body fluids, contact with non-intact skin or mucous membranes or when anticipated contact with chemical and chemotherapeutic agents*.

The use of sterile gloves are indicated when contact with susceptible sites or clinical devices where sterile conditions should be maintained such as urinary catheter insertion.

*Gloves used for the handling or in the administration of chemotherapeutic agents must be compliant with EN ISO 374-5:2016.

4.8.2 Apron / Gown

A single-use, fluid impervious gown or apron is to be worn by all healthcare workers when close contact with the patient, materials or equipment may lead to contamination of skin, uniforms or other clothing with infectious agents, or when there is a risk of contamination with blood, body substances, secretions or excretions.

An apron is appropriate when providing care requiring minimal contact with the patient or their environment.

Gowns and aprons must be changed between patients.

4.8.3 Face mask and eye protection

Face and eye protection reduces the risk of exposure to healthcare workers to splashes or sprays of blood and body substances. Procedures that generate splashes or sprays of blood and body substances, require either a face shield or a mask worn with protective eyewear.

Eyewear may include safety goggles or surgical masks with attached visors. A face shield may also be used as eye protection. Vision corrective glasses and contact lenses are not considered suitable protection from occupational exposures.

Masks should be changed between patients and when they become visibly soiled or wet.

4.8.3.1 Particulate Filter Respirators (PFR)

P2 and N95 PFRs are designed to achieve a very close facial fit with a seal around the nose and mouth for efficient filtration of airborne particles. All HCWs wearing a PFR must have undertaken a fit test, know the brand and size of PFR that they achieved a satisfactory fit to, and have access to that specific mask when required. Any PFR must be “fit-checked” each time it is worn, prior to entering a patient’s room. An effective seal may not be achieved when facial hair is present.

5. Transmission-based Precautions

Transmission-based precautions (TBP) are used in addition to standard precautions when patients are suspected or confirmed to be infected with agents transmitted by contact, droplet or airborne routes.

A summary of recommended precautions for specific infectious agents can be found in Appendix 3.

When transmission-based precautions are applied during the care of an individual patient, there is potential for adverse effects such as anxiety, mood disturbances, perceptions of stigma and reduced contact with clinical staff. Clearly explaining to patients why these precautions are necessary may help to alleviate these effects.

At AKG a coloured card system is utilised for informing HCWs of any TBP requirements before entry to and on exit from an affected persons room. The Nursing Shift Coordinator should ensure the coloured card(s) appropriate for the transmission-based precautions required is placed on the closed door of the isolation room. The card(s) should remain on the door for the duration of the patient’s isolation regardless of whether the patient is in the room.

PPE supplies are to be available outside the patient room or in the ante room if present. Gloves should be available inside the patient room to ensure compliance with gloves changes when performing hand hygiene as per the 5 Moments for Hand Hygiene. PPE must be donned prior to entering a patient room. Gown and gloves must be removed before exiting the patient room. Refer to Appendix 1 & 2 for PPE sequencing.



5.1 Contact precautions (Orange card)

Direct or indirect contact transmission of microorganisms during patient care is responsible for the majority of healthcare associated infections in patients and healthcare staff.

Room type	Single room with ensuite, door closed. Contact precautions (orange card) must be placed outside the room.
PPE	Long-sleeve disposable gown or plastic sleeveless gown.
Transfers	Inform the transporting staff and receiving department prior to transfer. Transport staff do not need to wear PPE unless directed by nursing or medical staff or if there is a risk of exposure to blood or bodily fluids. Hand hygiene must be performed after transporting the patient in line with the 5 Moment for Hand Hygiene.
Other	Non-essential staff and visitors to be kept to a minimum. Keep patient notes outside the room. Dedicate patient equipment for duration of precautions.

5.2 Droplet precautions (Green card)

Room type	Single room with ensuite, door closed. Droplet precautions (green card) must be placed outside the room.
PPE	Fit tested particulate filter respirator (PFR) e.g., N95/P2 with eye protection and fit-check performed after donning PFR prior to entering the patient's room and each time a new mask is put on.

	As per standard precautions, long-sleeve disposable gown or plastic sleeveless gown where there is risk of significant contamination of uniform.
Transfers	Inform the transporting staff and receiving department prior to transfer. Patient to wear surgical mask for transport, change to nasal prongs if on oxygen. Transport staff must wear a PFR, and other PPE as directed by nursing staff or if there is a risk of exposure to blood and bodily fluids. Hand hygiene must be performed after transporting the patient in line with the 5 Moment for Hand Hygiene.
Other	Non-essential staff and visitors to be kept to a minimum. Keep patient notes outside the room Dedicate patient equipment for duration of precautions.

5.3 Airborne precautions (Blue card)

Room type	Negative pressure isolation room with anteroom Airborne precautions (blue card) must be placed outside the room. Negative Pressure Isolation Rooms are located: • Emergency Department, Bay 46 • Canning Ward, Room 2 • ICU, Room 3
PPE	Fit tested particulate filter respirator (PFR) e.g., N95/P2 with fit-check performed after donning PFR prior to entering the patient's room and each time a new mask is put on. Eye-protection should be worn when undertaking aerosol generating procedures As per standard precautions, long-sleeve disposable gown or plastic sleeveless gown where there is risk of significant contamination of uniform.
Transfers	Inform the transporting staff and receiving department prior to transfer. Transport staff do not need to wear PPE unless directed by nursing or medical staff or if there is a risk of exposure to blood or bodily fluids. Hand hygiene must be performed after transporting the patient in line with the 5 Moment for Hand Hygiene.
Other	Non-essential staff and visitors to be kept to a minimum. Keep patient notes outside the room Dedicate patient equipment for duration of precautions.

5.4 Additional information

Guidance on the prioritisation of single rooms and potential cohorting opportunities for inpatients on transmission-based precautions are discussed in the AKG [Single Room Prioritisation and Cohorting of Inpatients on Transmission-based Precautions Procedure](#).

Crockery and utensils used by patients under TBP do not require containment and should be treated in the same manner as those used for non-infectious patients (i.e. washed in a dishwasher). Disposable crockery and utensils are not necessary.

6. Compliance monitoring

Compliance against this policy will be monitored by the Preventing and Controlling Infections Committee.

7. Policy context

The following documents are required to give effect to this policy:

- Legislative
 - Work Health and Safety Act 2020
- EMHS
 - [Infection Prevention and Management Policy](#)
 - [Waste Management Policy](#)
 - [Personal Protective Equipment Policy](#)

8. Supporting information

The following documents support the implementation of this policy:

- HDWA
 - [WA Health infection prevention and control policies and guidelines](#)
- AKG
 - [Hand Hygiene Policy](#)
 - [Occupational Exposure to Blood and Body Fluids Procedure](#)
 - [Blood and Body Fluid Spills Management Procedure](#)
 - [Cleaning and Disinfection of Reusable Clinical Non-Invasive Equipment Procedure](#)
 - [Aseptic Technique Procedure](#)
 - [Safe Handling and Waste Management of Hazardous Medications Guideline](#)
 - [Linen Management Policy](#)
 - [Single Room Prioritisation and Cohorting of Inpatients on Transmission-based Precautions Procedure](#)

9. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 3 – Preventing and Controlling Infections Standards
 - Action 3.04 - Partnering with consumers
 - Actions 3.06, 3.07, 3.08, 3.09 - Standard and Transmission-based precautions

- Action 3.10 - Hand hygiene
- Actions 3.13, 3.14 - Clean and safe environment
- Action 3.16 - Infections in the workforce

10. References

1. National Health and Medical Research Council (2019). Australian Guidelines for the Prevention and Control of Infection in Healthcare. <https://www.nhmrc.gov.au/health-advice/public-health/preventing-infection>
2. Government of WA, Department of Health (2023) [Personal Protective Equipment in Healthcare Facilities Policy](#)
3. Shaban et al. (2024) Healthcare-Associated Infections in Australia. Elsevier.

11. Policy contact

Enquiries relating to this policy may be directed to:

Title: Clinical Nurse Specialist
 Department: Infection Prevention & Management
 Email: ahsinfection.controlnurses@health.wa.gov.au and
Mariah.Wright@health.wa.gov.au

12. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	16 01 2026	16 01 2029	Early full review, significant changes to throughout document.

13. Authorisation

This policy has been authorised and issued by:

Authorised by	Russell Colyer-Cockburn - Director Nursing and Midwifery.
Authorisation date	15 01 2026
Published date	16 01 2026

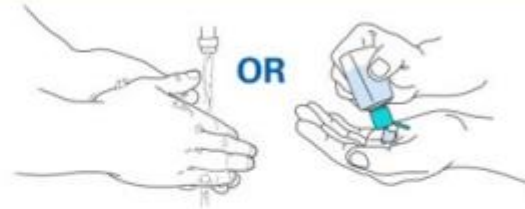
Appendix 1: Sequence for donning PPE

Sequence for putting on PPE

Put on PPE before patient contact and generally before entering the patient room

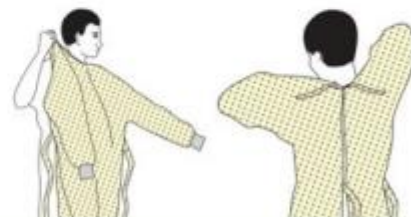
1. Perform hand hygiene

- Wash hands or use an alcohol-based hand rub.



2. Gown (if required)

- Fully cover torso from neck to knees, arms to end of wrists and wrap around the back.
- Fasten at the back of neck and waist.



3. Mask or particulate filter respirator (if required)

- Secure ties or elastic bands at middle of the head and neck.
- Fit check if using a particulate filter respirator.



4. Protective eyewear or face shield (if required)

- Place over face and eyes and adjust to fit.



5. Gloves (if required)

- Extend to cover wrist of gown.



Appendix 2: Sequence for doffing PPE

Sequence for removing PPE	
Begin PPE removal at doorway (for standard, contact and droplet precautions) or in the anteroom (for airborne precautions)	
1. Remove gloves - Outside of gloves is contaminated! - Grasp outside of gloves with opposite gloved hand; peel off. - Hold removed glove in gloved hand. - Slide fingers of ungloved hand under remaining glove at wrist. - Discard gloves in waste container.	
2. Perform hand hygiene Wash hands or use an alcohol-based hand rub.	
3. Remove gown - Gown front and sleeves are contaminated! - Unfasten ties. - Pull away from neck and shoulders, touching inside of gown only. - Turn gown inside out. - Fold or roll into a bundle and discard.	
Alternatively gloves and gown can be removed as one step. Then perform hand hygiene.	
4. Remove protective eyewear or face shield - Outside of eye protection or face shield is contaminated! - To remove, handle by head band or ear pieces. - Place in designated receptacle for reprocessing or waste container.	
Perform hand hygiene if the protective eyewear or face shield is contaminated.	
5. Remove mask - Front of mask is contaminated—DO NOT TOUCH! - Grasp bottom, then top ties or elastic and remove. - Discard in waste container.	
6. Perform hand hygiene Wash hands or use an alcohol-based hand rub immediately after removing PPE.	

Appendix 3: Recommended precautions and diseases list

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Adenovirus (Causing gastroenteritis symptoms)	Ingestion; contact with faecal material	Standard precautions Single room NOT required	Routine cleaning	Use Contact Precautions for incontinent persons or to control institutional outbreaks Precautions may be ceased 24 hrs after resolution of symptoms
Adenovirus (Causing respiratory symptoms)	Contact with respiratory secretions	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Precautions continue for duration of illness

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Anthrax (<i>Bacillus anthracis</i>) Cutaneous	Inoculation; person-to-person transmission rare	Standard precautions Single room NOT required Contact Precautions only if large amount of uncontained drainage of lesions.	Routine cleaning	Hand washing with soap and water preferable as alcohol does not have sporicidal activity
Aspergillosis (<i>Aspergillus</i> sp.)	Inhalation of airborne spores	Standard precautions Single room NOT required	Routine cleaning	Nil person to person spread Immunosuppressed patients are susceptible to aspergillosis infection and should be protected during hospital construction and renovation
Campylobacteriosis (<i>Campylobacter</i>)	Ingestion; contact with faecal material	Standard precautions Single room NOT required	Routine cleaning	Use Contact Precautions for incontinent persons or to control institutional outbreaks Precautions may be ceased 24 hrs after resolution of symptoms

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Candida auris	Likely contact	Contact precautions Single room with ensuite required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Refer to AKG Infection Prevention and Management for Multi-Resistant Organisms Guideline
Carbapenemase-producing Organism (CPO)	Contact	Contact precautions Single room with ensuite required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Refer to AKG Infection Prevention and Management for Multi-Resistant Organisms Guideline
Chickenpox (Varicella zoster virus)	Airborne droplets; direct contact with fluid in blisters or nasopharyngeal secretions	Airborne precautions Negative pressure isolation room required Use gloves +/- gown/apron if contact with body fluids likely	2 times a day cleaning of frequently touched surfaces Chlor-clean	Not to be attended by staff or visitors not immune to chickenpox Patient to remain isolated until all lesions dry and crusted over
Chlamydia pneumoniae		Standard precautions Single room NOT required	Routine cleaning	

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Chlamydia trachomatis	Contact with infected eye secretions; sexual transmission	Standard precautions Single room NOT required	Routine cleaning	
Cholera (<i>Vibrio cholerae</i>)	Ingestion; contact with faecal material	Standard precautions Single room NOT required	Routine cleaning	Use Contact Precautions for incontinent persons or to control institutional outbreaks Precautions may be ceased 24 hrs after resolution of symptoms
Conjunctivitis (Bacterial and viral)	Contact with infected eye secretions	Standard precautions Single room NOT required	Routine cleaning	Highly contagious, causes outbreaks in paediatric and neonate settings.
Clostridioides difficile (Formerly known as <i>Clostridium difficile</i>)	Ingestion; contact with faecal material	Contact precautions Single room with ensuite required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Commence stool chart Handwashing with soap and water preferred as alcohol does not have sporicidal activity Contact precautions must be continued for at least 48 hrs after diarrhoea has ceased

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
COVID-19 SARS-CoV-2 virus	Likely contact, droplet & airborne	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Refer to AKG Respiratory Virus Infection Prevention and Management Guideline
Creutzfeldt-Jakob Disease (CJD)		Standard precautions Single room NOT required	Routine cleaning	Refer to National CJD Infection Control Guidelines
Cryptosporidiosis	Ingestion; contact with faecal material	Standard precautions Single room NOT required	Routine cleaning	Use Contact Precautions for incontinent persons or to control institutional outbreaks Precautions may be ceased 24 hrs after resolution of symptoms
Cytomegalovirus (CMV)	Contact with infectious tissues	Standard precautions Single room NOT required	Routine cleaning	

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Dengue	Vector borne transmission	Standard precautions Single room NOT required	Routine cleaning	
Diphtheria (<i>Corynebacterium diphtheriae</i>) Cutaneous or pharyngeal	Contact with skin lesions / respiratory secretions	Contact precautions for skin lesions Single room with ensuite required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Duration of precautions until antimicrobial treatment ceased and when 2 cultures taken 24 hours apart are negative Patients with pharyngeal diphtheria to wear a surgical mask when outside of the room
		Droplet precautions including P2/N95 PFR for respiratory disease Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite		Standard precautions for non-toxin producing organisms.

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Escherichia coli (Shiga toxin-producing & enterotoxigenic E.coli)	Ingestion; contact with faecal material	Standard precautions Single room NOT required	Routine cleaning	Use Contact Precautions for incontinent persons or to control institutional outbreaks Precautions may be ceased 24 hrs after resolution of symptoms
Fungal Infections Dermatophytosis (Ringworm, tinea, Athlete's Foot)	Direct skin contact with humans or animals, or indirectly from contained items (e.g. shared towels), floors and soil	Standard precautions Single room NOT required	Routine cleaning	Can affect nails and skin of feet, body and head
German Measles (Rubella)	Droplet; contact with respiratory secretions	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Continue precautions until 7 days after onset of rash Non-immune and pregnant HCW should not attend patient

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Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Giardiasis Giardia	Contact with faecal material	Standard precautions Single room NOT required	Routine cleaning	Use Contact Precautions for incontinent persons or to control institutional outbreaks Precautions may be ceased 24 hrs after resolution of symptoms
Glandular Fever Epstein-barr virus (EBV) Infectious mononucleosis	Contact with saliva	Standard precautions Single room NOT required	Routine cleaning	
Hand, Foot and Mouth	Contact with fluid in blisters or faeces	Contact precautions Single room with ensuite required	2 times a day cleaning of frequently touched surfaces Chlor-clean	

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Haemophilus influenzae type b (HiB)	Direct and indirect contact with nose / throat secretions	Pneumonia in adults – Standard precautions Single room NOT required	Routine cleaning	Cease droplet precautions after 48 hours of effective antibiotic treatment <i>H. Influenzae</i> presents more commonly in children than in adults
		Invasive disease - Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	
Hepatitis A & E	Ingestion; contact with faecal material	Standard precautions Single room NOT required	Routine cleaning	Use Contact Precautions for incontinent persons Precautions may be ceased 24 hrs after resolution of symptoms
Hepatitis B, C, D & G	Contact with blood or body secretions	Standard precautions Single room NOT required	Routine cleaning	

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Herpes Simplex Virus (HSV 1 or 2) Skin, oral, genital or encephalitis	Contact with infected lesions	Standard precautions Single room NOT required	Routine cleaning	Cover vesicular lesions until dry and crusted
Herpes zoster (shingles)	Direct contact with fluid in blisters; respiratory secretions of patients with disseminated infections	Shingles – localised disease Standard precautions Single room NOT required	Routine cleaning	Contact precautions required for any lesions not covered or dressing does not contain exudate Cease isolation when lesions are dry and crusted Non-immune HCW should not attend patient
		Shingles - disseminated Airborne precautions Negative pressure isolation room required Use gloves +/- gown/apron if contact with body fluids likely	2 times a day cleaning of frequently touched surfaces Chlor-clean	Disseminated disease is characterised by a generalised eruption of more than 10-12 extra lesions outside the primary dermatomes For patients with disseminated infections, isolate for duration of symptoms Non-immune HCW should not attend patient

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Human Immunodeficiency Virus (HIV / AIDS)	Bloodborne & sexual	Standard precautions Single room NOT required	Routine cleaning	
Human Metapneumovirus	Likely contact, droplet & airborne	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Refer to AKG Respiratory Virus Infection Prevention and Management Guideline
Influenza	Likely contact, droplet & airborne	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Refer to AKG Respiratory Virus Infection Prevention and Management Guideline

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Legionellosis <i>Legionella longbeachae</i> <i>Legionella pneumophila</i>	Inhalation of contaminated soil / potting mix Inhalation of aerosolised contaminated water	Standard precautions Single room NOT required	Routine cleaning	
Leprosy <i>(Mycobacterium leprae)</i>	Contact with respiratory secretions	Standard precautions Single room NOT required	Routine cleaning	
Leptospirosis <i>(Leptospira sp.)</i>	Contact with blood and urine	Standard precautions Single room NOT required	Routine cleaning	
Lice (pediculosis)	Close person to person contact	Standard precautions Single room NOT required	Routine cleaning	Commence topical treatment immediately

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Listeriosis (<i>Listeria monocytogenes</i>)	Ingestion of contaminated food	Standard precautions Single room NOT required	Routine cleaning	
Malaria	Vector transmission	Standard precautions Single room NOT required	Routine cleaning	
Measles (rubeola)	Airborne spread; contact with infected respiratory secretions	Airborne precautions Negative pressure isolation room required Use gloves +/- gown/apron if contact with body fluids likely	2 times a day cleaning of frequently touched surfaces Chlor-clean	Can be removed from precautions 4 days after rash appears Inform Infection Prevention & Management of admission Not to be attended by non-immune HCWs

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Melioidosis (<i>Burkholderia pseudomallei</i>)	Inoculation; inhalation of contaminated soil and water	Standard precautions Single room NOT required	Routine cleaning	
Meningococcal infection (<i>Neisseria meningitidis</i>)	Close contact with respiratory droplets	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Cease isolation 24 hours after commencement of effective antibiotic therapy Post-exposure prophylaxis may be indicated
Methicillin-resistant Staphylococcus aureus (MRSA) Micro-Alert B Ciprofloxacin susceptible strain	Contact	Standard precautions Single room NOT required unless considered high risk ward / patient – see comments	Routine cleaning	Use contact precautions for patients with conditions increasing risk of transmission e.g., large wounds with high exudate, exfoliative skin conditions or if admitted to a higher risk area: ICU & Colyer (Surgical) Refer to AKG Infection Prevention and Management for Multi-Resistant Organisms Guideline

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Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Methicillin-resistant Staphylococcus aureus (MRSA) Micro-Alert C Ciprofloxacin resistant strain	Contact	Contact precautions Single room with ensuite required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Micro-alert C clones have increased antimicrobial resistance, virulence factors or high transmissibility in hospitals Refer to AKG Infection Prevention and Management for Multi-Resistant Organisms Guideline
Middle East Respiratory Syndrome Coronavirus (MERS-CoV)	Likely contact, droplet & airborne	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Refer to AKG Respiratory Virus Infection Prevention and Management Guideline
Mpox (Previously known as monkeypox and abbreviated as MPXV) Viral – Clade I	Direct contact	Contact precautions AND Airborne precautions with negative pressure isolation room required	2 times a day cleaning of frequently touched surfaces Chlor-clean	People are considered infectious for four days before the onset of symptoms and/or the appearance of rash and skin lesions Isolation may cease when no new lesions form and all existing lesions are dry and crusted See WA Health Mpox Quick Guide

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Mpox Viral – Clade II	Direct contact	Contact precautions AND Droplet precautions including P2/N95 PFR Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	People are considered infectious for four days before the onset of symptoms and/or the appearance of rash and skin lesions Isolation may cease when no new lesions form and all existing lesions are dry and crusted See WA Health Mpox Quick Guide
Mumps	Droplet; contact with respiratory secretions	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Cease isolation 5 days after onset of swelling. Exposed non-immune people should be considered infectious from 12 - 25 days after exposure, with or without symptoms
Mycoplasma pneumoniae	Likely contact, droplet & airborne	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Droplet precautions required for duration of illness

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Norovirus	Contact	Contact precautions Single room with ensuite required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Commence stool chart Contact precautions must be continued for at least 48 hrs after diarrhoea has ceased Handwashing with soap and water preferred as alcohol does not have sporicidal activity Additional Droplet precautions if patient's symptoms include vomiting
Parainfluenza	Likely contact, droplet & airborne	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Refer to AKG Respiratory Virus Infection Prevention and Management Guideline
Parvovirus B19 infection (Erythema infectiosum)		Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Precautions continue for duration of illness

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Pertussis (whooping cough) (<i>Bordetella pertussis</i>)	Droplet, Airborne	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Cease isolation 5 days after commencement of antibiotic therapy; or 21 days after the onset of symptoms if no treatment given
Plague (<i>Yersinia pestis</i>)		Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Cease isolation 48 hours after initiation of effective antibiotic therapy
Poliomyelitis	Ingestion; contact with faecal and oral secretions	Contact precautions Single room with ensuite required		

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Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Q Fever (<i>Coxiella burnetii</i>)	Inhalation of infected animal tissue, contaminated soil or dust	Standard precautions Single room NOT required	Routine cleaning	
Rabies / Australian Bat Lyssavirus		Single room precautions Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Contact precautions required if unable to contain secretions
Respiratory Syncytial Virus (RSV)	Likely contact, droplet & airborne	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Refer to AKG Respiratory Virus Infection Prevention and Management Guideline

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Rhinovirus	Likely contact, droplet & airborne	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Refer to AKG Respiratory Virus Infection Prevention and Management Guideline
Ross River Virus	Vector transmission	Standard precautions Single room NOT required	Routine cleaning	
Rotavirus	Ingestion; contact with faecal material and respiratory secretions	Contact precautions Single room with ensuite required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Commence stool chart Contact precautions must be continued for at least 48 hrs after diarrhoea has ceased Additional Droplet precautions if patient's symptoms include vomiting

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Rubella (see German Measles)				
Salmonella <i>Salmonella paratyphi</i> <i>Salmonella typhi</i>	Contact with faecal material	Standard precautions Single room NOT required	Routine cleaning	Use Contact Precautions for incontinent persons or to control institutional outbreaks Precautions may be ceased 24 hrs after resolution of symptoms
Scabies (<i>Sarcoptes scabiei</i>)	Skin to skin contact or from infested fomites	Contact precautions Single room with ensuite required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Ensure bed linen/clothing is changed after patient has completed treatment Maintain precautions until 24 hours after treatment commenced
Severe Acute Respiratory Syndrome (SARS)	Likely contact, droplet & airborne	Contact precautions AND Airborne precautions with negative pressure isolation room required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Fit check to be performed prior to entering room Precautions continue for duration of illness + 10 days after resolution of fever, provided respiratory symptoms are absent or improving

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Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Shigella	Ingestion; contact with faecal material	Standard precautions Single room NOT required	Routine cleaning	Use Contact Precautions for incontinent persons or to control institutional outbreaks Precautions may be ceased 24 hrs after resolution of symptoms
Smallpox (variola)	Contact with respiratory secretions, contaminated fomites	Contact precautions AND Airborne precautions with negative pressure isolation room required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Precautions continue for duration of illness until all scabs have crusted and separated (3-4 weeks) See Smallpox CDNA National Guidelines for Public Health Units
Staphylococcus aureus Impetigo	Contact	Standard precautions Single room NOT required	Routine cleaning	
Staphylococcus aureus Extensive exposed wounds / burns	Contact	Standard precautions Single room NOT required	Routine cleaning	Contact precautions if unable to contain wound, continue precautions until wound has healed

Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Streptococcus pyogenes Invasive Group A Streptococcal infections (iGAS)	Bacterial	Droplet precautions including P2/N95 PFR Use gloves +/- gown/apron as per standard precautions if contact with body fluids likely Single room with ensuite	2 times a day cleaning of frequently touched surfaces Chlor-clean	Transmission-based precautions to remain until 24 hours after effective antibiotics
Tuberculosis <i>Mycobacterium tuberculosis</i> Pulmonary or laryngeal	Airborne	Airborne precautions Negative pressure isolation room required Use gloves +/- gown/apron if contact with body fluids likely	2 times a day cleaning of frequently touched surfaces Chlor-clean Close room for 1 hour prior to discharge cleaning	Refer to AKG Tuberculosis Infection Prevention & Management Guideline
Tuberculosis <i>Mycobacterium tuberculosis</i> Extrapulmonary	Airborne	Standard precautions Single room NOT required	Routine cleaning	

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Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Typhoid (<i>Salmonella typhi</i>)	Faecal-oral route; ingestion of contaminated water and food	Standard precautions Single room NOT required	Routine cleaning	Transmission by direct person-to-person contact can occur but is rare Contact precautions required for incontinent persons and must be continued for at least 48 hrs after diarrhoea has ceased
Vancomycin resistant Enterococcus (VRE)	Contact	Contact precautions Single room with ensuite required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Refer to AKG Infection Prevention and Management for Multi-Resistant Organisms Guideline
Varicella zoster virus (see Chickenpox)				
Viral haemorrhagic fevers (VHF) Crimean-Congo Ebola Lassa Marburg	Airborne; contact with blood and body fluids	Contact precautions AND Airborne precautions with negative pressure isolation room required	2 times a day cleaning of frequently touched surfaces Chlor-clean	Contact Clinical Microbiologist See Viral Haemorrhagic Fever Response Plan for Western Australia

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Disease / Organism	Transmission route	Required precautions	Cleaning requirements	Duration of precautions and additional comments
Worms (including tapeworm, pinworm, threadworm)	Ingestion	Standard precautions Single room NOT required	Routine cleaning	
Zika Virus	Mosquito bite; vertical transmission from mother to baby; sexual transmission	Standard precautions Single room NOT required	Routine cleaning	